



Vancouver Clinic

Enterprise Imaging RFP & Selection Project

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CONFIDENTIAL INFORMATION

This document is the property of Health Innovative Solutions, LLC (H-I-S) and Vancouver Clinic (VC), is strictly confidential, and contains information intended only for the person or individuals to whom it is transmitted. With the receipt of this information, the recipient acknowledges and agrees that: (1.) this document is not intended to be distributed, and if distributed inadvertently, will immediately be returned to H-I-S / VC; (2.) the recipient will not reproduce, divulge, or distribute this confidential information, in whole or in part, without the express written consent of H-I-S / VC; (3.) all of the information herein will be treated as confidential material by the recipient with no less care than that afforded to its own confidential material.



HEALTH INNOVATIVE SOLUTIONS, LLC

Mission

The mission of Health Innovative Solutions is to be a change agent looking to bring new and innovative data capture and reporting solutions to improve the quality of care and to reduce costs.

Our Vision

To transform the collection, presentation, and utilization of institutional data for the betterment of the patient and healthcare provider enhancing Health and Wellness for all.



INVITATION

Vancouver Clinic (VC) hereby invites you (here after referred to as the "bidder", "vendor", or "vendor partner") to submit a proposal to furnish a complete Imaging Solution supporting the clinic and its patients across its Southern Washington state and Northern Oregon locations which include Multi-Specialty Clinics, Urgent/Immediate Care, and Ambulatory Surgery Centers. Prior to submitting a proposal, the vendor should consider the requirements outlined in this document and the required commitment of resources to respond in a professional manner.

Vancouver Clinic chose the invited list of vendors by its Imaging selection committee which consists of both internal and external resources, each with a different take on the needs of the organization. This group of vendors selected for inclusion by the team is based on industry performance, recommendations, and alignment with Vancouver Clinic, and the clinic will accept no other bids. The bidders invited to participate in this process are (in alphabetical order):

- Fujifilm Healthcare Solutions
- GE Healthcare
- Intelera Medical Systems
- Merge, a Merative Company
- Optum, Change Healthcare Stratus Imaging
- Sectra
- Visage Imaging

Per the request of Vancouver Clinic, the bidders may not conduct direct contact with Vancouver Clinic from this point forward until a selection is made. All negotiations are conducted through the Health Innovative Solutions, LLC Consultant / Program Manager. Representatives of Vancouver Clinic will attend the bidder's conference, and the Program Manager consults the team on questions submitted throughout this process.

In the end, Vancouver Clinic desires an adaptive, technology-driven, and financially viable solution supporting their enterprise imaging needs, traditionally defined as PACS, CPACS, and VNA.

Whether each invited entity chooses to bid on this project or not, Vancouver Clinic and Health Innovative Solutions would like to thank you for your participation in this process. We realize that there is an initial effort involved, but both organizations are committed to overseeing this process in a thoroughly professional manner.



RFP BID PROCEDURES

This RFP represents a definition of specific user requirements and an invitation for recipients to submit a response addressing such requirements. Issuance of this Request for Proposal and Design (“RFP”) and your preparation and submission of a response does not require Vancouver Clinic to award a contract to any bidder. Only the execution of a written contract will oblige Vancouver Clinic in accordance with the terms and conditions contained in such contract.

PROPOSAL REQUIREMENTS

Kick-Off Conference

The kick-off conference call for the project occurs in accordance with the detailed schedule published along with this document. A representative of the bidder’s organization must be present at the Microsoft Teams Video Conference to comply with the bidding process.

[Click Link to Conference](#)

WHEN IT IS TIME, JOIN THE MICROSOFT TEAMS MEETING HERE:

Join the meeting now

Meeting ID: 255 830 834 582

Passcode: HR6im6JB

No representatives should be onsite at this kick-off conference.

Acknowledgement & Intent to Propose Solution

After reviewing this document and attending the Kick-Off Conference, any Vendor who intends to provide a bid for Enterprise Imaging must fill out the [Appendix B: Acknowledgement & Intent to Propose Solution](#) form included herewith. Receipt must occur in accordance with the detailed schedule published along with this document.

[Corporate Stability and Financial statements](#)

As part of the process of supplying the [Appendix B: Acknowledgement & Intent to Propose Solution](#) form, Vancouver Clinic requests each vendor submit Corporate Stability and Financial Statements. For public companies, each vendor must submit the most recent Annual Report and any Securities & Exchange Commission (SEC) filings along with a Corporate Stability statement. For private companies, each vendor must submit Audited Financial Statements along with a Corporate Stability statement. This should be completed within **ten (10) business days** of the vendor supplying the form.

Intent to Withdraw

After reviewing this document and/or attending the Kick-Off Conference, any potential Vendor may choose to “No Bid” or “Withdraw” from the Bid Process. To clarify your position, you must fill out the [Appendix C: Intent to Withdraw](#) form included herewith. This includes an acknowledgement that everyone connected



with your organization/company destroyed all electronic and/or printed copies of the document. Receipt of the document aligns with the detailed schedule published along with this document.

Failure to Submit One of the Above Forms

Any Vendor who does not submit the *Appendix B: Acknowledgement & Intent To Propose Solution* form or the *Appendix C: Intent to Withdraw* form will be considered a **Non-Compliant Partner of Vancouver Clinic**. The result of this may be exclusion from this process as well as all the other bid processes currently active or under development by Vancouver Clinic.

Bidder Video Conferences

To provide each bidder with a unique opportunity to consult with Vancouver Clinic on their response, ask questions and discuss specifics regarding their products, and provide insights into the proposed relationship and value of their solution, Vancouver Clinic is holding dedicated Bidder Video Conference with each of the entities.

Using the published schedule, a specific date and time for each bidder is established. The meeting slot is chosen via Random Draw at the RFP Kick-Off Conference.

- A two (2) hour slot is provided and up to one (1) hour of dedicated time can be made available ad-hoc for clarifications. The team suggests the vendor develop an agenda which includes the following:
- Detailed Question & Answer Session on RFP and Response
- Detailed Technical Discussion targeted at Imaging Informatics & Information Technology Resources
- Proposed Relationship

The agenda should include durations for each session so the appropriate resources can attend. Vancouver Clinic does not wish for team members that do not have a direct interest in the topic discussed having to participate in sessions not relevant to their role if they choose.

Vancouver Clinic requires each vendor to discuss currently available solutions. Any consideration of additional technology components, including futures discussed or shown in addition to the current released versions must be available for General Release by **December 31, 2025**.

Legal Contracting Process

To ensure Vancouver Clinic and the finalists can execute a contract upon final selection and in accordance with our overall objectives, Vancouver Clinic will enter direct contract negotiations with the selected bidder based on the detailed schedule published along with this document. As part of the RFP process, Vancouver Clinic requests identification of resources used in the Legal Contracting Process in the response. This allows for informing all Legal Representative(s) of the selection and ensure that the process can begin immediately.

At this time, Vancouver Clinic believes that there will be a single finalist for the solution. As a result, Vancouver Clinic is setting a Preliminary Contract Negotiation Schedule. Each Vendor should ensure that their team can meet this schedule if selected as a finalist in accordance with the detailed schedule published along with this document.

Forecasting

For the benefit of the bidders and understanding Vancouver Clinic's post-selection timeline, the following is provided.



Description	Start Date	Completion Target
Bidder Selection Date	Thursday, May 1, 2025	Thursday, May 1, 2025
Legal Contracting Process Drive to Executable Contract	Friday, May 2, 2025	Friday, May 16, 2025
Vancouver Clinic Executive Contract Review	Monday, May 19, 2025	Monday May 19, 2025
Vancouver Clinic Budgeting Process (Internal Process)	Tuesday, May 20, 2025	Monday, June 27. 2025
Budget Approval 2026 Letter of Intent Delivered	Monday, June 30, 2025	Monday, June 30, 2025
Contract Execution & Initial Payment	Monday, January 5, 2025	Monday, January 5, 2025
Phase II: Implementation Kick-Off	Monday, January 19, 2026	Monday, January 19, 2025

Official Notification & Signing

It is the intention of Vancouver Clinic to execute all components of this project in accordance with the detailed schedule published along with this document.

Questions

The Vendor should address all questions concerning this RFP in writing using electronic mail ONLY to the Program Team at the e-mail address dreynolds@health-innovative.com. The team does not accept questions in any other form (hard copy, fax, or telephone) unless previously cleared by the Program Manager, Mr. David Reynolds, using the electronic mail process. The final date for submitting questions is in accordance with the detailed schedule published along with this document.

Authorized Contact Information

To ensure streamlined communications, Vancouver Clinic requests the vendor provide detailed contact information in accordance with the detailed schedule published along with this document. The form is after the Appendix in this document and titled *Appendix D: Detailed Contact Information*.

Proposal Validity Date

Your proposal, with authorized signatures, constitutes an offer which shall remain valid for a period of one hundred eighty (180) days from the bid due date.

Pricing

Each proposal must include complete unit pricing, including detailed part numbers for the systems outlined in this document. Vancouver Clinic does not expect to incur any additional charges for components specified herein unless the scope of the work is in addition to this document. Pricing should be a complete solution. Proposals MUST include the information requested in the following sections, in the order and detail needed to satisfy each item shown. The vendor must consider the quotations submitted in reply to this request binding.

Confidentiality

The attached specification and any information verbally gathered might contain confidential information pertaining to Vancouver Clinic and its partners. Vancouver Clinic prohibits all information from use or disclosure in any other form other than in connection with your bid.



Compliance with Requirements

The Vendor shall confirm that in all respects the proposed solution conforms to all specifications, terms and conditions incorporated and made a part of this RFP. The Vendor will ensure that responses utilize the same order and use the same numbering conventions as used in this RFP to facilitate the evaluation process. The Vendor should ensure inclusion of all required information in this response and state acceptance of conformance or otherwise, to every point in this RFP. The team does not accept neutral comments such as "noted" as a compliant statement. Any failure and/or incompleteness in this respect may result in rejection of your response.

Completeness of Response

By virtue of the submission of a response to this RFP, the bidder warrants that the requirements of this RFP have been read and understood and represents that the delivery of the products and support contracts specified in this RFP shall in no way Vancouver Clinic to pay any additional costs to the vendor for the provision of the products and support services, other than those so noted and presented in the response to this RFP.

Right of Rejection

Vancouver Clinic reserves the right to accept or reject any or all responses to this RFP and to enter discussions and/or negotiations with more than one qualified supplier at the same time, should such action be in the best interests of Vancouver Clinic.

Right of Incorporation

Your response to this RFP does not constitute an offer to do business with Vancouver Clinic. Vancouver Clinic may, at its option, incorporate all or any part of this RFP and your response to this RFP in the contract.

Confidentiality of Bidder Information

Vancouver Clinic requests the vendor provide data about the organization and the products. In the event any data supplied to Vancouver Clinic is confidential, please note such confidentiality in a separate letter. Vancouver Clinic agrees that for a period of one (1) year it shall take the same steps to protect the confidentiality of such information, as it takes in connection with information about itself and its products that it deems confidential. The vendor may not deem confidential information about the vendor or the products if: 1) already known to the parties, 2) already in the public domain, or 3) disclosed to the parties by a third party having a right to make such disclosure. In no event, without the prior written consent of Vancouver Clinic, should you reveal to the parties any data which you deem a trade secret, because the parties' obligation of confidentiality as set out in this paragraph may not be sufficient to protect the same.

Compliance with Laws

By virtue of submitting its signed proposal, the bidder acknowledges and attests that the proposed components and support services conform to and comply with all applicable federal, state, county and municipal laws, codes, and regulations.

Software Licensing

In the event of a contract award, the vendor will grant Vancouver Clinic a non-exclusive, perpetual license to use any software provided because of this RFP as specified in the terms and conditions of any contract or



software license agreement executed by Vancouver Clinic and the vendor. Any proprietary software developed by the vendor for use in the recommended system will become the property of Vancouver Clinic.

Use of Names

In connection with this RFP the bidder shall not use the name of Vancouver Clinic or any of their individual subsidiaries or affiliates in any publications or public relations announcements without the express written consent of Vancouver Clinic prior to such publication or announcement. Vancouver Clinic individually reserves the right to review and approve all press related copy and may withhold consent for release of such copy, with or without cause, at its sole discretion.



RFP BASE REQUIREMENTS

This RFP represents a definition of specific requirements and an invitation for recipients to submit a response addressing such requirements. Issuance of this Request for Proposal (“RFP”) and your preparation and submission of a response does not commit Vancouver Clinic or any of either party’s subsidiaries to award a contract to any bidder. Only the execution of a written contract will oblige Vancouver Clinic, in accordance with the terms and conditions contained in such contract.

Specifications & Information

Use of Subcontractors

These specifications cover required services, service level agreements, contracts, and requirements necessary for Vancouver Clinic to execute a contract for their Enterprise Imaging. The vendor must include contracts that may require the employ of Subcontractors to provide meet the requirements of the contract. Vancouver Clinic reserves the right to approve all subcontractors. Should the Vendor use subcontractors today for these services, The Vendor must include subcontractor information in accordance with Section 5.0 of this document.

Mounting

To provide the services to Vancouver Clinic, the Vendor may need to install equipment onsite. When necessary, the Vendor shall supply all mounting hardware, equipment, and supports required to mount the equipment in its already installed data center cabinets, except as otherwise noted.

Completeness

The Vendor should provide all other items called for in this specification, except as otherwise noted, in accordance with the conditions of the contract.

Use of Terms

The RFP uses certain terms such as "shall, provide, complete, start up, cutover" in parts of these specifications. This does not indicate that the items shall not be complete or available or in any way less than complete at the time of delivery.

Workable Solution

It is the intent of the specifications to provide a complete, workable solution ready for Vancouver Clinic. The vendor must include any item not specifically called for in the specifications but normally required to conform to the intent of the contract.

Documentation

These specifications are detailed services and performance specifications. Actual implementation of the processes shall be as indicated in detailed implementation documents and, where necessary, detailed drawings and installation documents. Installations and details indicated on the drawings and installation documents shall govern if they differ from the specifications and the obligation of the vendor is to identify such differences at the time of submitting the bid.

Use of Abbreviations & Terminology

The vendor may use the abbreviations and terms shown in [Appendix A: Abbreviations and Terminology](#) within the RFP Response, implementation process documents, drawings, and specifications:



Submittals

File Formats

Submit all documents in a printed form and file format compatible with Microsoft Office 365, Microsoft Office 2019, Microsoft Project 2019, Microsoft Visio 2019, Adobe Acrobat PDF, or newer versions of these software packages.

Manufacturer Specifications

Submit manufacturer specifications data for all equipment furnished if required to deliver the Enterprise Imaging solution detailed in the response.

Product Information

Vendor's product data shall consist of illustrations, standard schedules, performance charts, instructions, brochures, diagrams, and other information furnished by the Vendor to illustrate the capabilities of their solution when providing a complete solution to Vancouver Clinic.

Bill of Materials

During preparation of a final contract, the Vendor's Representatives, proposed Manufacturer Representatives, and Service Providers / Sub-Contractors must submit for review and acceptance a list of all required material and equipment for delivering your proposal to a production environment. This technical review requires all parties to signify in writing all required components that exist in the final documents.

Incorporation of Submittals

The submittal list shall include all material, systems and equipment as listed herein.

Site Visits & Schedule Coordination

- The Vendor may wish to visit the site of the proposed work to identify any existing conditions that may affect the work, as the Vendor shall be responsible for any assumptions in regard thereto.
- The Vendor should coordinate all site visit activities through Vancouver Clinic Program Manager, Mr. David Reynolds.
- Vancouver Clinic shall conduct the overall administration for the program. The Vendor shall be responsible for establishing and maintaining communication with the Program Manager in this regard.
- Within five (5) business days following contract award, the Vendor shall prepare and issue a delivery schedule to Vancouver Clinic. The Vendor shall from that point forward be responsible for coordinating that schedule with Vancouver Clinic or its representatives on a continuous basis.

Coordination of the Work

- The Vendor is to refer to and meet the project schedule for the implementation at all locations. The schedule aligns in harmony with the requirements of Vancouver Clinic.
- Carefully check space requirements and the physical requirements of the areas of work to ensure that all material required to deliver the services outlined in the RFP can be installed in the spaces allotted. This includes outlining requirements for Data Centers and Physician / Technologist Workspaces.
- Identify in advance all items of work that require access to Vancouver Clinic or to arrange for access.



Execution

Staffing

- The Vendor should provide sufficient supervisory work force to maintain efficient performance of the Vendor's responsibilities. The Vendor shall designate in writing to Vancouver Clinic a primary local contact for resolution of problems, coordination, additions, changes, and alike. The Vendor's representative shall have full authority to represent the Vendor in making decisions and resolving issues in accordance with the contract. The Vendor as part of this bid response shall supply a resume of the primary representative and technical engineers assigned to the project.
- The Vendor shall use only a skilled, accredited, experienced, and reliable work force and shall discontinue the involvement of anyone associated on this project upon request by Vancouver Clinic.

Organization of Work

Vendors should take into consideration incurring any additional costs to meet the requirements of the contract within limited or restricted periods. The vendor must include this cost, if any, in your response. The Vendor shall furnish promptly to these and to any other individuals or entities involved in the delivery of a working system all information relating to the project which they may require and cooperate with them to secure the harmony necessary to the best interest of the project.

Contract Initiation Requirements & Term Requirements

Vancouver Clinic has provided a list of requirements (guidance) for the contract between Vancouver Clinic and the Vendor.

Upon determination that the Vendor is a finalist in the process, Vancouver Clinic and the Vendor will move to the Contract Negotiation and Final Selection Phase. Vancouver Clinic and the Vendor agree to work on good faith basis to produce legally binding language complete with Service Level Agreements, Liability Agreements, Initiation Terms, Payment Terms, Severability, and other contractual commitments to meet the timelines outlined in this document.

VANCOUVER CLINIC IS IN NO WAY BOUND TO SELECTING THE PROPOSING VENDOR OR EXECUTING A CONTRACT.

Vancouver Clinic requests that the Vendor adhere to the following structures around startup, initial payments, and structure moving forward:

- A. The vendor must outline all Consulting Dollars and associated Payment Structures separately from the Product and Implementation Costs.
- B. For Product and Implementation Costs, the bidding entity shall provide a fixed cost for implementation. This cost is a one-time cost and will be paid out under the following terms: A Zero - 80 - 20 schedule; zero percent (0%) down, eighty percent (80%) at completion of detailed testing plan, and twenty percent (20%) percent when Vancouver Clinic signs off that all associated systems are in Production for thirty (30) days of uninterrupted activity – hereby referred to as “the Acceptance Period”.
- C. The first year of all support, maintenance and associated contracts does not start until the Acceptance Period is complete and upon execution of the sign-off process.



- D. Vancouver Clinic requires that the Vendor complete all training and documentation before the thirty (30) day acceptance period begins.

Training Requirements and Resource Recommendations

The Vendor, as part of assembling a complete, sole source solution, should identify in detail the training requirements and resource recommendations to enable Vancouver Clinic to take over management of the solution at the end of the project period. This should include the following:

- A. Recommended Training Courses and Tracks
- B. Recommended Certifications
- C. Recommended Reading Materials
- D. Recommendations as to the Staffing Levels needed to maintain the complete solution on a Seven (7) Day a Week, Twenty-four (24) Hours-a Day, Three Hundred Sixty-Five (365) Day a Year basis. This should include onsite, remote, and/or Smart-Hands resources.

Non-Discrimination / Equal Opportunity

The Vendor during the Bid Process as well as if the entity is selected and contracts executed must agree to the following:

- A. In the hiring of employees for the performance of work under the Contract or any subcontract, Contractor and its subcontractors will not, by reasons of race, color, religion, sex, age, disability, military status, national origin, or ancestry; discriminate against any citizen of this state in the employment of a person qualified and available to perform the work to which the Contract relates.
- B. Contractor, its subcontractors, and any person acting on behalf of Contractor and its subcontractors will not, in any manner, discriminate against, intimidate, or retaliate against any employee hired for the performance of work under the Contract on account of race, color, religion, sex, age, disability, military status, national origin or ancestry.

As part of the RFP, the Bidder will sign and submit *EXHIBIT A: Non-Discrimination / Equal Opportunity Affidavit*.

Bid and Performance Bond Requirements

At present, Vancouver Clinic **does not require** Bid Bonds or Performance Bonds for this process. If during the evaluation process, Vancouver Clinic leadership determines one or both bonds is necessary, the Program Manager plans to incorporate the requirements in an Addendum to the RFP.

Bid Bonds

The Vendor MUST indicate their willingness to provide a Bid Bond if requested.

Performance Bonds

The Vendor MUST indicate their willingness to provide a Performance Bond if requested. Should a Performance Bond be requested, the Vendor shall furnish the Performance Bond to Vancouver Clinic within ten (10) calendar days of contract award notice and prior to contract commencement.



DETAILED CLINICAL SPECIFICATIONS

The following sections detail Vancouver Clinic's desires in gaining a complete solution provided by the Bidding Entity in a turnkey, managed offering to support our Enterprise Imaging environment.

1.0 Clinical Requirements and Associated Technical Components

Standard Imaging Protocols

Vancouver Clinic outlined the following requirements for vendors who wish to provide a response to this Request for Proposal and Design. In responding to this section of the document, the vendor must provide detailed responses to each line item listed below. The design requirements are based around what Vancouver Clinic identified as key features for a system that would be ideal to meet the needs of Vancouver Clinic.

- 1.1 The system must be able to support Radiologists' primary reading functions regardless of reading location in an identical, high-speed workflow.
 - 1.1.1 Example for **1.1**: Performance should be identical whether the reading of the study is on-premises at a clinical location or from a dedicated station at home.
 - a Provide minimum network performance parameters to include upload/download speeds and maximum allowable latency to provide a seamless reading experience
 - 1.1.2 Currently, Vancouver Clinic utilizes a Radiologist based in the State of Alaska. Please indicate how Vancouver Clinic and your solution supports this example location.
- 1.2 The ideal system provides Workflow Orchestration natively within the environment enabling the organization to establish flexible methodologies for Reading Assignments, Workload Leveling, and Specialty Prioritizations.
- 1.3 The system should be able to manage studies encompassing large datasets with the same speed as standard datasets.
- 1.4 The ideal system must be able to show in the study list the time at which the study became available in the Enterprise Imaging solution.
- 1.5 The ideal system must have customizable hanging protocol toolsets easily configured by the Radiologist and/or system administrator. The hanging protocol toolsets must follow the Radiologist regardless of the location where they login to the system
 - 1.5.1 Vendor will provide a foundation set of hanging protocols for all modalities including comparisons for at least one prior and cross modality.
- 1.6 The ideal system should have the ability to view more than a single modality in the same viewing session. For example, when the physician is reading a CT, they desire the ability to view the associated X-Ray and MRI in the same viewing session.
- 1.7 The ideal system must be able to have customizable settings, based on study type and Radiologist, to pull relevant priors based on quantity and/or dates of service.
- 1.8 The ideal system should have the ability to view more than one patient at any given time (e.g., multiple patient exams open in a session).
- 1.9 The ideal system should allow for the Radiologist to read and dictate two (2) normal chest X-Rays in sixty (60) seconds or less (one [1] in thirty [30] seconds).



- 1.10 The ideal system must load exams and the associated dictation window simultaneously in a rapid, clinically relevant fashion. Currently, the solution which will operate with the Enterprise Imaging solution is Microsoft Nuance® PowerScribe™ One.
 - 1.10.1 The dictation solution should be a self-contained Enterprise Imaging driven solution allowing direct dictation. Vancouver Clinic does not have and currently does not believe a RIS will be installed in conjunction with this project.
- 1.11 The ideal system should allow for an automated integration of comparison studies to allow for workflow efficiency and provide key integration (as appropriate) with the designated structured reporting solution and/or Microsoft Nuance® PowerScribe One.
- 1.12 The system must allow for easily workable doctor's worklist in all aspects of the system, including the web-based / teleradiology interface.
 - 1.12.1 The worklists should be easy to filter and incorporate the ability to incorporate site-based and skill-based protocols.
- 1.13 The ideal system must have user-friendly case management tools and functions available to technologists. This should include built in QA functionality.
- 1.14 The ideal system must have user friendly system management tools and functions available to technologists and system administration staff.
- 1.15 The ideal system must have automatic comparison of cases regardless of the architecture utilized.
 - 1.15.1 The overall speed for studies located in any associated long-term archive must be rendered as if storage was in the primary Enterprise Imaging regardless of location or study size.
 - 1.15.2 If the Enterprise Imaging solution presented utilizes any sort of pre-fetch mechanism, please document how this is executed and ensure that all parameters within 1.14 are met.
- 1.16 The ideal system should support a "Disruption Workflow." For example, if the case is expedited, the Radiologist must be able to quickly access the prioritized case without having to close the current case.
- 1.17 The ideal system must support direct integration with Epic Corporation EHR and load the associated patient chart when the physician selects the patient from an Enterprise Imaging Solution-driven Worklist.
- 1.18 The ideal system must support direct HL7® integration, HL7® FHIR® integration, and pull relevant patient reports into the Enterprise Imaging (not PDF-based reports) for use in comparisons.
 - 1.18.1 These relevant patient reports should not require loading of the images to view the report.
 - 1.18.2 The ideal system vendor should be participating in the Epic Showroom FHIR Platform for integration based on Epic standards and to include specifications available at open.epic.com.
- 1.19 The ideal system should include support the ability for "Paging" Ordering Physicians | Critical Test Result Management in an automated fashion. This should be driven through the EPIC platform from messages generated in the Enterprise Imaging solution.
- 1.20 The ideal system should allow for worklists to be easy to filter and incorporate the ability to site-based and skill-based protocols.
- 1.21 The ideal system must have automatic linking and cross-referencing of images. (e.g., Multiplanar point references).



- 1.22 The ideal system must allow for measurements to auto-populate based on study type.
- 1.23 The ideal system must ensure that all annotations made by Vancouver Clinic staff or Radiologists save AS-IS.
- 1.24 The ideal system should support Nuclear Medicine and Diagnostic Ultrasound color and dynamic images.
- 1.25 The system must support review of all modalities from the same workstation.
- 1.26 The ideal solution would drive efficiencies in Structured Reporting. Today, Vancouver Clinic utilizes the following and desires to incorporate, where possible, embedded solutions provided and supported by the Enterprise Imaging solution.
 - 1.26.1 Replacement of MagView
 - 1.26.2 Replacement of LungView
 - 1.26.3 Replacement of Obstetrics Digisonics
 - 1.26.4 Replacement of Mirada
 - 1.26.5 Equivalent functionality to existing TeraRecon platform
 - 1.26.6 Equivalent functionality to existing Syngovia platform
- 1.27 The ideal system should allow for Hot Key Commands (i.e., control functions using buttons or numeric keypads). Recommended devices / supported devices should be documented in your response.¹
- 1.28 The vendor should describe how their solution accomplishes Peer Review.
 - 1.28.1 The ideal system should support Peer Review functions natively including “auto peer review.”
 - a Should this solution not exist inside the solution proposed, the Vendor must provide insights and recommendations as to which products / solutions can be integrated into the proposed solution to accomplish this requirement.
 - b Should this solution not exist inside the solution proposed, the Vendor should provide the detailed interface requirements, products, and professional services costs for integrating the recommendations outlined in (a) into the workflows of the proposed solution.
 - c Should this solution not exist inside the solution proposed, the Vendor should provide detailed interface requirements and specifications for integrating the recommendations outlined in (a) into the Epic Corporation EHR environment as appropriate.
- 1.29 The system must support high-speed access and comparisons across multiple patients for use in Tumor Boards and by non-Radiologists regardless of location.
- 1.30 The system should allow for easy export of images to JPEG and DICOM formats for use by Vancouver Clinic personnel in various presentations and case reviews.
- 1.31 Regardless of the architecture of the solution, the system must allow for retrieval of any images stored in the Enterprise Imaging solution and export or share the information in Native Format and with Full Fidelity when required.

¹ For specifics related to Women’s Imaging, see specific requirements specified.



- 1.31.1 Example: If images are compressed in the solution, the solution must support full and immutable exports as if they had been directly sent by the modality.
- 1.32 The system must allow for easy access to patient demographics regardless of the interface utilized.
- 1.33 The system must allow saving of annotations made by the modality's workstation to the Imaging Platform solution.
- 1.34 The system must easily integrate with the current Epic Corporation EHR Platform. Describe in detail the interface method(s) employed for ADT, transcription, and results reporting.
- 1.35 The desired solution incorporates all functionality in an Enterprise Imaging Solution. Should the solution utilize DIOCM, the bidding entity MUST provide details on how the solution maintains "Lockstep" between the Production Interpretation environment and the Archive components including details on Study Finalization and operating when Vancouver Clinic is operating in an Epic Downtime mode.
- 1.36 The system must allow for easy transfer of data from the modality to the Enterprise Imaging solution and issue a DICOM confirmation receipt to the operator upon success.
- 1.37 The solution should incorporate Tracking Tools / Dashboards in real-time to assess productivity of all associated resources (e.g., Radiologists, Technologists, Cardiologists, and alike).
- 1.38 The system should use Windows 11 for workstations and use standard workstation components available from recognized industry manufacturers. Vancouver Clinic owned Workstations exist as entities in Active Directory.
- 1.39 The Vendor MUST provide minimum and recommended workstation specifications for Radiologist workstations, Mammography workstations (if different from the Radiologist workstations), Tech workstations, and Cardiology workstations.
 - 1.39.1 The Vendor must detail Multiple Monitor management, layout, and support for running the applications provided in the solution along with PowerScribe™ One, and other software applications (e.g., Epic Hyperdrive).
- 1.40 The system must incorporate detailed logging functions, either natively or via integration into Vancouver Clinic's Splunk Platform for the following:
 - 1.40.1 System Performance with Thresholds configured for event triggers (e.g., Latency)
 - 1.40.2 Current Application Performance with Thresholds configured for event triggers.
 - 1.40.3 DICOM related requests and errors with Thresholds configured for event triggers.
 - 1.40.4 Failed User Access Authentication Requests with Thresholds configured for event triggers.
 - 1.40.5 Break the Glass events.
 - 1.40.6 Monitoring of HL7® Interfaces with event triggers (Cloverleaf discussion)
 - 1.40.7 Modality Monitoring indicates and triggers based on Modality failures when sending DICOM and non-DICOM studies to the environment with Thresholds configured for event triggers.



- 1.41 Vancouver Clinic currently utilizes Microsoft® Nuance PowerShare™ access, exchange, and viewing of images to and from outside entities. The system must incorporate this platform into the overall solution.²
 - 1.41.1 Describe process for managing outside studies and integration into existing MRN numbering and automated near match patient identities
- 1.42 The system must allow burning of cases to CD/DVD for distribution to referring physicians with an “embedded light viewer” or web browser interface and launcher. Storage of images on the disc should be in native format.³
- 1.43 The bidding entity should provide insights into direction around the above list and future directions for the platform.

Women’s Imaging Protocols

Currently, Vancouver Clinic operates a specific workflow solution, Hologic SecurView DX with dedicated hardware / software, for interpretation of all Breast Imaging Modalities / Studies. Based on the solution provided in your response, the organization desires to remove the components of the specialty Diagnostic Workstations and Structured Reporting solution with an all-in-one solution provided as part of the overall Enterprise Imaging solution. For this to be considered and deployed as part of the going forward strategy, the following requirements must be met.

- 1.43 The solution must be able to support all functions of the Breast Imaging solution in a remote / work-from-home capable solution with identical functionality as that available onsite at Vancouver Clinic locations.
- 1.44 An equivalent or better workflow solution to that of the SecurView DX workstation with the following features:
 - 1.44.1 An Intuitive and ergonomic solution which boosts the reading experience for these specialty radiologists. This includes the use of plug-and-play “specialty” hardware like that shown in *Figure 1: Hologic SecurView DX Workstation User Interface / Keypad* below:



Figure 1: Hologic SecurView DX Workstation User Interface / Keypad

² Vancouver Clinic only accepts images via PowerShare or through importation into PowerShare.

³ Vancouver Clinic currently utilizes PowerShare for this function and intends to continue.



- a The number of devices should be included in the RFP Response.
 - b A scroll wheel, which boasts single-touch support for navigation through tomosynthesis images, allowing the reader to scroll or cine.
 - c Single-click access to functions they commonly use, eliminating the need to hunt for the review tools they find most helpful.
 - d Minimal keystrokes for all breast imaging modalities.
- 1.44.2 Ability for Radiologists to set their individual preferences for scrolling mode, cine speed, default slab thickness, and a default initial slice.
- 1.44.3 Capacity to customize reading and functional settings that fit the needs of their specialty and helps manage workloads.
- 1.44.4 Personalization of Report Flows for any clinical situation, allowing radiologists to set up workflows fitting their preferences.
- 1.44.5 Support for multimodality image display and software support incorporating Digital Breast Tomosynthesis, Breast Ultrasound, and Breast MRI Images.
- 1.44.6 Mammography Reporting: The vendor must supply how Mammography Reporting, which currently is incorporated into the Hologic's workflow, is accomplished in the proposed solution.
 - a If the solution is native to the platform, please provide detailed information.
 - b If the solution is provided by a third party, please incorporate all recommendations and components in the response.
 - i Indicate if this component is supported and maintained by your organization as part of the Enterprise Imaging solution.
- 1.44.7 Migration considerations: In moving from Hologic's environment to the proposed Enterprise Imaging solution, the bidding entity should note that conversion from Hologic's SCO format to BTO format may be required.
- 1.44.8 The vendor must provide a workstation specification that uses Industry-Standard components as the base platform.
- 1.44.9 The vendor should provide in detail any specific requirements related to subnet configurations at the server and workstation layer, and any specific network interface configurations required by the system.
- 1.44.10 The solution must be able to support all functions of the Breast Imaging solution in a remote / work-from-home capable solution with identical functionality as that available onsite at Vancouver Clinic locations.

Echocardiography Protocols

Currently, Vancouver Clinic operates a dedicated CPACS environment utilized for Echocardiography and the associated reporting only. Moving forward, Vancouver Clinic desires to move all associated workflows in the Enterprise Imaging solution being proposed. The following requirements are indicated below:

- 1.45 The solution should provide Echocardiography structured reporting and integration to external registries



Point-of-Care Ultrasound

Currently, Vancouver Clinic utilizes Point-of-Care Ultrasound which is not read by Radiologists. Moving forward, Vancouver Clinic desires to move all associated workflows in the Enterprise Imaging solution being proposed. The following requirements are indicated below:

- 1.46 The solution must be able to support Point-of-Care Ultrasound with a dedicated worklist and auto sign-off of completed exams.

Future Clinical Workflows

As the intent of this process to implement an Enterprise Imaging strategy, options for incorporating future modalities and workflows into the solution being presented are ideal.

- 1.47 The bidding entity should provide insights into which area may be included. Use the list below as a starting point and add to this as appropriate.
 - Endoscopy
 - Wound Care
 - Video (multiple areas)
 - And more

Clinical Relationship

- 1.48 The vendor should provide a detailed statement as to the proposed relationship with Vancouver Clinic's Clinical teams and the methods used to maintain a solid partnership over the next five (5) years.

Study-based Licensing.

Vancouver Clinic's primary model is for studies / exams created internally. However, using PowerShare the organization receives outside studies regularly for comparisons. Vancouver Clinic does not intend to archive ANY studies received from outside entities in perpetuity excluding external mammography studies which will be maintained in a long-term archive.

- 1.49 The vendor must provide a licensing model for all inside-created studies based on the volumes supplied for new studies.
- 1.50 The vendor should provide a one-time migration cost for migrating all existing studies to the new platform.
- 1.51 The vendor must provide a licensing model for all outside studies with the understanding that these studies will be utilized in a ninety (90) day First-In, First-out (FIFO) model and that these have a significantly low or no cost related to studies generated in-house.



CLOUD-BASED COMPONENTS/SOLUTIONS

To support the clinical requirements outlined in DETAILED CLINICAL SPECIFICATIONS, as stated previously, Vancouver Clinic prefers a Native Cloud / Solution-as-a-Service. Vancouver Clinic provides a base level of requirements for any part of the solution that incorporates these components. This is to be deployed in a fully Operational Expense model.

2.0 Public Cloud Providers

- 2.1 The Bidding Entity must provide the name of the proposed Public Cloud Provider (where applicable) as part of the response. The following providers currently are considered options by Vancouver Clinic:
- Amazon Web Services (AWS)
 - Microsoft Azure (the Microsoft Cloud platform)
 - Google Cloud Platform (GCP)
- 2.1.1 If no Public Cloud provider is utilized in your solution, please indicate that separately. The Bidding Entity will need to provide additional information on their infrastructure in their response.
- 2.2 The Bidding Entity must provide information on locations within the Public Cloud environment chosen to support Vancouver Clinic's proposed solution with the following included:
- 2.2.1 Geography
 - 2.2.2 Regions
 - 2.2.3 Zones
 - a Availability Zones
 - b Local Zones

3.0 Data Center supporting Cloud Environments

When utilizing Public Cloud providers, many of the points listed below are part of the solutions provided. For Private Cloud environments, information in these areas tends to be lacking through publicly available means. As a result, the Bidding Entity is asked to indicate the following to the items below for Public Cloud instances: **Provided by Public Cloud provider**. When the solution is part of the Vendor's Private Cloud, the details below MUST be completed in full.

- 3.1 All Physical Data Centers where the proposed *Cloud-Based Components/Solutions* operate supporting Vancouver Clinic are located within the United States of America.
- 3.2 All data and configurations required to operate and support the proposed *Cloud-Based Components/Solutions* is not stored, replicated, backed up, or any in way moved to locations outside the United States of America without prior written consent of Vancouver Clinic.
- 3.3 Uptime Institute must certify all Physical Data Centers® for proposed *Cloud-Based Components/Solutions* utilizing the following criteria:
- 3.3.1 Tier III or Tier IV Certification of Data Center Design Documents or equivalent
 - 3.3.2 Tier III or Tier IV Certification of Constructed Data Center Facility or equivalent
 - 3.3.3 Tier III or Tier IV Certification of Data Center Operational Sustainability or equivalent



4.0 Security and Privacy for Cloud Environments

As part of a response where the Vendor and its partners propose *Cloud-Based Components/Solutions*, the Vendor MUST document the following in their response:

- 4.1 Document how the application(s) within the *Cloud-Based Components/Solutions* provide Data Encryption in Transit and At Rest.
- 4.2 Document of how often Third-Party Penetration Tests performed on the *Cloud-Based Components/Solutions*. Please note this is different from Vulnerability Scanning.
- 4.3 Document on how access to the *Cloud-Based Components/Solutions* supports data transit security.
- 4.4 Document on whether the entity providing the *Cloud-Based Components/Solutions* performs regular SSAE 16 SOC 2 Audits on the proposed environment.
 - 4.4.1 Indicate when the latest SSAE 16 SOC 2 Audit occurred.
 - 4.4.2 Provide (if available) with the latest SSAE 16 SOC 2 Report.
- 4.5 Document the network, system, and web application vulnerability management process for the proposed *Cloud-Based Components/Solutions*.
- 4.6 Document the Security Controls that enforce Separation of Duties for the proposed *Cloud-Based Components/Solutions*.
- 4.7 Document the Security Controls in place for Endpoint Protection on systems used by developers, system administrators, and others supporting the *Cloud-Based Components/Solutions*.
- 4.8 Detail how those supporting the *Cloud-Based Components/Solutions* authenticate to it and the monitoring and logging of such access occurs.
- 4.9 Describe integration with third-party SOC providers.
- 4.10 Document the *Cloud-Based Components/Solutions* two-factor authentication strategy.
- 4.11 Document the provider(s) of the *Cloud-Based Components/Solutions* ability to prevent, detect, and respond to intrusions, including processes in place to do so.
- 4.12 Document the provider(s) of the *Cloud-Based Components/Solutions* information security organizational structure. Please include internal and external personnel, roles, and responsibilities.
- 4.13 Document the methods and support for E-Discovery for information stored within the *Cloud-Based Components/Solutions*. Include the options available to Vancouver Clinic to provide record requests (if needed).
- 4.14 Details of how the *Cloud-Based Components/Solutions* provides support for Data Loss Prevention (DLP). Describe all the available options to prevent data leaks or misuse.
- 4.15 Document the security measures incorporated into the *Cloud-Based Components/Solutions* to maintain compliance with the following standards:
 - 4.15.1 HIPAA
 - 4.15.2 HITRUST
 - 4.15.3 TECFA
 - 4.15.4 21st Century Cures Act
 - 4.15.5 Any additional applicable standards



5.0 User and Administrative Access to Cloud Environments

As part of a response where the Vendor and its partners propose *Cloud-Based Components/Solutions*, the Vendor MUST certify that User and Administrative Access to the environment meets the following criteria:

- 5.1 User Level Access to the *Cloud-Based Components/Solutions* MUST integrate into Vancouver Clinic's Entra ID environment to include enterprise single sign on, passthrough authentication where appropriate, role based security controls and authentication parameters (password expiration, account status, and alike).
- 5.2 Administrative Level Access to any component in the *Cloud-Based Components/Solutions* MUST utilize Two-Factor or Multi-Factor Authentication. Vancouver Clinic's current Two-Factor Authentication / Multi-Factor Authentication platform is Cisco Duo, and the solution should directly integrate as such.
- 5.3 Vancouver Clinic requires logging and tracking of all attempted User Level Access or Administrative Level Access to any component of the *Cloud-Based Components/Solutions* for Successful and Unsuccessful Attempts.
- 5.4 When the number of Unsuccessful Login Attempts is equal to or greater than three (3) attempts to the same ID within a five (5) minute period MUST trigger a Security Alert to the Support Team from the provider of the *Cloud-Based Components/Solutions* and to Vancouver Clinic's Service Desk.
- 5.5 Vancouver Clinic requires granting of controlled User Level Access and Administrative Level Access via IP Access Controls and via Vancouver Clinic's Public IP Address Space.

6.0 Scheduled Events within Cloud Environments

As part of a response where the Vendor and its partners propose *Cloud-Based Components/Solutions*, the Vendor MUST document the following in their response:

- 6.1 Document the Patching, Update, and Upgrade schedules for the next twenty-four (24) months for the *Cloud-Based Components/Solutions*.
- 6.2 Document the process for Patching, Updates, and Upgrades for *Cloud-Based Components/Solutions*.
- 6.3 Document System Availability during performance of Patching, Updates, and Upgrades for *Cloud-Based Components/Solutions*. Describe whether not the Uptime Guarantee factors in System Availability.

7.0 Incident Management within Cloud Environments

As part of a response where the Vendor and its partners propose *Cloud-Based Components/Solutions*, the Vendor MUST document the following in their response:

- 7.1 Document the detailed processes for Incident Management for the *Cloud-Based Components/Solutions*. Provide detail aimed at demonstrating the ability of the provider(s) to manage incidents regarding software, hardware, applications, interfaces, transport, security and other relevant processes or components that comprise the Vancouver Clinic environment.
- 7.2 Document the method for reporting incidents when discovered through the support or technical resources providing the *Cloud-Based Components/Solutions*.
- 7.3 Document on how reporting incidents occurs when discovered by Vancouver Clinic resources.
- 7.4 Document escalation paths for incidents and how escalation is determined:



- 7.4.1 Time-Based/Automatic Escalations
- 7.4.2 *Cloud-Based Components/Solutions* Support Staff/Management Escalations
- 7.4.3 Client Initiated Escalations
- 7.4.4 Document protocols for Incident Management and Data Protection in the event of a Security-based Incident.
- 7.4.5 Document the Availability and Schedules for Support Resources for the *Cloud-Based Components/Solutions*.
 - a Vancouver Clinic requires 7 x 24 x 365 availability to support resources for the products and solutions incorporated in the response. We desire to know the number of Support Resources assigned throughout the daily cycle.

8.0 Cloud Access Security Broker (CASB)

As part of a response where the Vendor and its partners propose *Cloud-Based Components/Solutions*, the Vendor MUST document the following in their response:

- 8.1 Document whether the provider and solution provided for the *Cloud-Based Components/Solutions* can be integrated into the existing Cloud Access Security Broker (CASB) environments in place today and the associated history of using this technology within the proposed environment (e.g., Cisco Cloudlock, Forcepoint).

9.0 Scheduled Events within Cloud Environments

As part of a response where the Vendor and its partners propose *Cloud-Based Components/Solutions*, the Vendor MUST document the following in their response:

- 9.1 Document whether the provider for the *Cloud-Based Components/Solutions* provides connectivity to Vancouver Clinic's network or whether Vancouver Clinic is responsible for providing connectivity to the provider.
- 9.2 Document anticipated connectivity requirements necessary to provide access to the *Cloud-Based Components/Solutions* with no single point of failure between Vancouver Clinic and the proposed environment.

10.0 Scheduled Events within Cloud Environments

Whether delivered as a Shared Instance or Private Cloud Instance, Infrastructure Lifecycle Management of all associated components is key to maintaining the long-term viability of the environment. Vancouver Clinic wants to understand the mechanisms associated with this. As a result, the Vendor MUST document the following:

- 10.1 At the Physical Data Center locations where the *Cloud-Based Components/Solutions* deployed, document how lifecycle management of data center equipment is managed and whether there are costs that may be passed down associated with Lifecycle Management.
- 10.2 Document the impact to Vancouver Clinic when there are changes to the environment related to Lifecycle Management and if they are separate and distinct from *9.0 Scheduled Events within Cloud Environments*.
- 10.3 Document performance thresholds or other mechanisms that may initiate an early review of the data center equipment as it relates to Lifecycle Management and factor sizing into the equation.



11.0 Event Notification

Whether delivered as a Shared Instance or Private Cloud Instance, Event Notification (where applicable) is key. Please provide information on the following:

Shared Instance / Public Cloud

11.1 The Bidding Entity must describe how event notification is incorporated as part of the solution.

Private Cloud / Private Instance

11.2 The system must incorporate detailed logging functions for integration into Vancouver Clinic's Splunk Platform for the following technical and security components in a deployment:

11.2.1 Operational Functions: Storage Monitoring with Thresholds for event triggers

11.2.2 Operational Functions: Software / Services Events & Failures

11.2.3 Operational Functions: Hardware Failures / Virtual Hardware Failures

12.0 Premise-based Components.

Some components of the solution may incorporate a limited number of "Premise-based Components." The components would be placed inside of Vancouver Clinics two (2) colocation facilities. It is intended for these to be Fully Managed by the Vendor. Additionally, the vendor MUST provide information on the following items related to these Premise-based components.

12.1 The vendor must provide detailed specifications for implementing the solution in a two (2) data center, Active-Active or Active-Passive configuration with a less than thirty-minute Recovery Point Objective (RPO) and Recovery Time Objective (RTO).

12.2 The vendor should indicate the deployment methodology for these premise-based components:

12.2.1 Vendor supplied appliances / servers.

12.2.2 Virtual Servers / Storage supplied by Vancouver Clinic

12.3 The vendor must provide information as to the ownership of Operating System (OS) updates and upgrades. For example, in the Microsoft environment, Microsoft releases multiple "Windows Update" files. Vancouver Clinic needs to understand the requirements of their information technology staff or that there is no requirement for them to manage these changes, that the vendor will provide and install these updates in a timely manner.

12.4 The vendor must describe in detail your organization's staging parameters and how you plan to integrate with the Virtual Server Environment.

12.5 The vendor should include a detailed breakdown of all the hardware requirements from a processor, memory, drive size, drive volume, and disk controller specifications for all servers.

12.6 The vendor should provide a system with an application-level uptime guarantee of 99.95% and recommend hardware components with a 99.999% uptime level.

12.7 The vendor should indicate the life cycle of the current architecture.

12.8 The vendor should document the Mean Time between Failure (MTBF) of all components.

12.9 The vendor should document the list of all bugs and their fixes over the last 18 months.

12.10 The vendor should document the complete list of all approved hardware vendors.



- 12.11 The vendor should document whether they are the sole provider of the software or where they have embedded third-party software into the solution including the list of third-party vendors.
- 12.12 All Physical Data Centers where the proposed *Cloud-Based Components/Solutions* operate supporting Vancouver Clinic are located within the United States of America.
- 12.13 All data and configurations required to operate and support the proposed *Cloud-Based Components/Solutions* is not stored, replicated, backed up, or any in way moved to locations outside the United States of America without prior written consent of Vancouver Clinic.
- 12.14 Uptime Institute must certify all Physical Data Centers® for proposed *Cloud-Based Components/Solutions* utilizing the following criteria:
 - 12.14.1 Tier III or Tier IV Certification of Data Center Design Documents or equivalent
 - 12.14.2 Tier III or Tier IV Certification of Constructed Data Center Facility or equivalent
 - 12.14.3 Tier III or Tier IV Certification of Data Center Operational Sustainability or equivalent
- 12.15 The system's architecture should be client / server based or server-side rendering-based and use open-system architecture such as Microsoft Windows or Linux-based servers.
- 12.16 The ideal system should utilize Microsoft Entra ID platform allowing single sign-on with the rest of Vancouver Clinic's security structure.
- 12.17 The system should also include security certificates for WEB based users.
- 12.18 The ideal system should integrate using Vancouver Clinic's existing Domains, Policies, and Procedures. No dedicated authentication structure is acceptable.
 - 12.18.1 To further clarify, the vendor should provide in their response an acceptable, integrated user, group, administrative structure example for consideration and any platform limitations (e.g., Group Policy).
 - 12.18.2 The vendor should provide in their response any settings that deviate from CIS recommendations.
- 12.19 User Level Access for all external access MUST utilize Two-Factor or Multi-Factor Authentication. Vancouver Clinic's current Two-Factor Authentication / Multi-Factor Authentication platform is Cisco Duo, and the solution should directly integrate as such. The solution is utilized across all portions of the environment.
- 12.20 When needed to complete functions within the environment ONLY, the ideal system should utilize Microsoft Office365 and no other client-related software for documents, spreadsheets, presentations, or electronic mail.



CLINICAL TRAINING AND CREDITS; SUPPORT STAFF

For Vancouver Clinic's Providers, learning the new workflows and performance improvement is critical to their ongoing success. Additionally, the requirement to learn best practices on the associated products is imperative. As a result, Vancouver Clinic added these additional requirements.

13.0 Clinical Training and Credits

- 13.1 The Vendor must incorporate Semi-Annual training and tuning sessions with the Radiologists, Mammographers, and Cardiologist to incorporate best practices, improve hanging protocols, and examine efficiencies of the workflows. Please incorporate all associated costs for such training.
 - 13.1.1 If available with the above, please provide information as to the mechanism for earning Continuing Medical Education Credits (CME) for this training.
- 13.2 The Vendor must incorporate training associated with 13.1 into each planned upgrade of the platform as appropriate.

14.0 Support Staff

- 14.1 Vancouver Clinic wishes to clearly identify the requirements for defining the Job Description and Training Requirements for internal support resources for the proposed Enterprise Imaging solution. The vendor must provide a detailed Job Description including, but not limited to, Educational Requirements, Job History Requirements, Experience Requirements, and a Detailed List of Duties. Additionally, should the vendor indicate a resource can be trained to meet the requirements, please detail the training requirements to achieve competency and/or technical certification.



REGULATORY SECURITY REQUIREMENTS

In December 2020, the National Institute of Standards and Technology (NIST) division of the U.S. Department of Commerce and the National Cybersecurity Center of Excellence released NIST Special Publication 1800-24L Securing Picture Archive and Communication System (PACS): Cybersecurity for the Healthcare Sector⁴.

Under the direction of Vancouver Clinic's CISO, the following requirements are set forth and should be incorporated in the Vendor's response.

Enterprise Imaging Solution Security Controls

Goal

Vancouver Clinic would like to implement a multi-layered security strategy for the proposed Enterprise Imaging solution. The evaluation of the proposed solution will focus on whether the Enterprise Imaging application suite has the following capabilities and whether the vendor can work with Vancouver Clinic based on the following approach. Should the vendor become a finalist in the process, Vancouver Clinic's Security Team can conduct a more detailed vendor security risk assessment.

15.0 Detailed Security Requirements

The Vendor must respond in detail to each of the following items:

- 15.1 The file preamble is cleansed when receiving or importing DICOM files, but nevertheless antivirus software and application whitelists are used on the DICOM viewers receiving the files.
- 15.2 The integrity of images is secured with digital signatures, but nevertheless the network transmission of the images uses public-key certificates to authenticate authorized image sources.
 - 15.2.1 Can one or more digital signatures be applied to a complete DICOM image or parts thereof, and then to be embedded in the DICOM header, which makes sure that the digital signature is always stored and transmitted as part of the signed image or document, together with the certificate of the signer, which permits a validation of the signature by any system that receives the image?
 - 15.2.2 Is there the ability for the system to have a digital watermark? (Watermarking describes a process whereby information (such as identity information or a digital signature) is "hidden" in the image pixel data in the form of high frequency information ("snow") that is invisible to the human eye but recognizable by an algorithm that specifically checks the presence of a digital watermark.)
 - 15.2.3 Can a digital watermark be used for authentication purposes? (The main advantage of digital watermarks over digital signatures is that watermarks are difficult to remove from the image, whereas digital signatures can easily be removed.)
- 15.3 Images are de-identified, when possible, but nevertheless encryption is used for the transmission.
 - 15.3.1 Image de-identification: It is required that images be de-identified before they are transmitted to a third party or submitted to a database.
 - 15.3.2 Transport security: Describe TLS and encryption used to exchange medical images.

⁴ The publication can be found at <https://nvlpubs.nist.gov/nistpubs/SpecialPublications/NIST.SP.1800-24.pdf>



- 15.3.3 Selective encryption of DICOM Header: In some cases, it is desirable to selectively encrypt only those attributes of the DICOM header that contain patient identifying information instead of applying a transport security scheme. Does system have this capability?
- 15.3.4 Full encryption of DICOM images: An alternative to either a transient transport encryption or a selective encryption of DICOM header fields is obviously the full encryption of DICOM images including both header and image pixel data. Does system have this capability?
- 15.4 Network communication in the Enterprise Imaging network is migrated to encrypted and authenticated transmission, but nevertheless the network is secured with firewalls and network segmentation.
- 15.5 The Enterprise Imaging network is secured with firewalls and network segmentation, but nevertheless an intrusion detection system is deployed to detect breaches of the network security.
- 15.6 Access to images in the Enterprise Imaging is controlled by user specific access rights, but nevertheless an audit trail is deployed that allows to analyze, “who did what.”
- 15.7 Antivirus software and whitelists are deployed where possible to prevent ransomware attacks, but nevertheless a secure, remote backup of the database is maintained.
- 15.8 A “Security Data Workflow Diagram” depicts infrastructure, architecture, data exchange / transmission, data storage, data entry – access points, encryption, and alike. Does the Enterprise Imaging vendor have this diagram? If yes, please provide.



SUPPORT AND BUSINESS REQUIREMENTS

The following specifications outline Vancouver Clinic's Support and Business Requirements.

16.0 Specifications

- 16.1 The vendor must provide support 7 days a week, 24 hours a day, 365 days per year, with a 30-minute response. The vendor must be able to provide support services via a connection through the Internet and Vancouver Clinic's SecureLink environment. Vancouver Clinic will not allow utilization of other solutions on any user devices.
- 16.2 The vendor must provide resumes and client evaluation for all field service engineers assigned to the account.
- 16.3 The vendor must provide documented escalation procedures for phone support and field service engineers including contact information.
- 16.4 The vendor should provide access to product specific user groups and show evidence of how the vendor incorporates user input on potential changes to the system into planning and development processes and demonstrate a pattern of rolling these changes into production. The vendor should also show evidence of newsletters to users.
- 16.5 The vendor must document the number of software revisions, patches, hot-fixes, and full releases released over the past twenty-four (24) months, with detail provided including release notes for the past twelve (12) months.
- 16.6 The vendor should document how testing of the product occurs, including under real network load conditions such as those experienced in a hospital organization the size of Vancouver Clinic operating a comparable radiology department.
- 16.7 The vendor should document how to conduct onsite testing for revisions and update releases and an example of how implemented. This should indicate roll-back procedures and ways to limit the impact on the business of major changes. This must be in conjunction with Vancouver Clinic's Change Management Process.
- 16.8 The vendor must describe the training requirements to the organization for all levels, both the pre- and post-installation training requirements for users, administrators, and IT staff.
- 16.9 The vendor must document the bandwidth requirements for LAN Access and WAN Access including the minimum, maximum, and average by application.
- 16.10 The vendor should provide evidence of a disaster recovery protocol / plan for the systems recommended and show examples of successful implementations and assessments.
- 16.11 The vendor must allow Vancouver Clinic local IT staff administrative control for all systems.
- 16.12 The vendor should document the Organizational Structure including Sales, Technical, Administration, and Development teams including contact information.
- 16.13 The vendor should demonstrate a detailed Project Plan for the implementation and administration of the Enterprise Imaging Solution.
- 16.14 The vendor should describe the data and administration reporting capabilities of the Enterprise Imaging solution.
- 16.15 The vendor must provide a provision regarding data stored on their system for seamless extraction without the loss of critical patient data or annotations at the termination of the contract(s).



- 16.16 The vendor must provide FDA audit capability and HIPAA compliance statements.
- 16.17 The vendor should define the process for modality compatibility confirmation (interface confirmation).
- 16.18 The vendor must submit DICOM, FHIR, and HL7® conformance statements on electronic media along with their written RFP response (to reduce the paper volume).
- 16.19 The vendor should submit a detailed document outlining the standard Warranty and Conditions. This should be separated from any defined maintenance contracts.
- 16.20 The vendor must be willing to submit quarterly inspection tests by Vancouver Clinic personnel or their contractors to ensure that all provisions of the contracts are still in force.
 - 16.20.1 Pen Test
 - 16.20.2 Annual Risk Assessment
- 16.21 The vendor should submit detailed information on the Market Strength of the products outlined in their Enterprise Imaging solution.
- 16.22 The vendor should submit Corporate Stability and Financial Stability statements.
- 16.23 The vendor should outline their sales record, and the number of recent alike hospital installs including full references for the products outlined in their Enterprise Imaging solution.
- 16.24 The vendor should detail the Research and Development spending as a percentage of Enterprise Imaging sales for the products outlined in their Enterprise Imaging solution.
- 16.25 The vendor should provide a Vision Statement for their organization and/or Enterprise Imaging Solution.
- 16.26 The vendor should provide a statement as to what the differences are between their products and their chief rivals.



PRICING

Vancouver Clinic requires the vendor to break down the pricing into the following line items. When it comes to the hardware portion of this project, Vancouver Clinic proposes that the vendor work with the Program Manager to ensure that it complies with Vancouver Clinic's established standards and verify all products recommended are acceptable prior to submitting the final bid.

17.0 Component Breakdown

Please fill in the table below and subsequently provide detailed backup to this, including part numbers, in your response.

Note: Cloud-Based solutions must include Ingress/Egress Charges based on Current and Projected Volumes.

17.1 Direct Purchase / Capital Costs (NRC)

DESCRIPTION	MSRP/RETAIL	DISCOUNTS ⁵	BUYBACKS	FINAL COSTS
Components & Software				
Servers				
Storage Platforms				
Network Connectivity				
Network Components				
Storage Network Components				
Diagnostic Workstations				
Support Contracts				
Servers				
Storage Platforms				
Network Connectivity				
Network Components				
Storage Network Components				
Diagnostic Workstations				
TOTAL FOR SECTION				

17.2 Operational Costs: Cloud-Based or Software-as-a-Service (ARC)

DESCRIPTION	MSRP/RETAIL	DISCOUNTS	BUYBACKS	FINAL COSTS
Components & Software				
Servers				
Storage Platforms				
Network Connectivity				
Network Components				
Storage Network Components				
Diagnostic Workstations				
Support Contracts				
Servers				
Storage Platforms				
Network Connectivity				
Network Components				
Storage Network Components				
Diagnostic Workstations				
TOTAL FOR SECTION				

⁵ Should not include "Registration Discounts." Manufacturers have been informed that these are not to be included at this time – only applied at the final contract time as part of a contingency budget.



17.3 Consulting / Services Costs (NRC)

DESCRIPTION	MSRP/RETAIL	DISCOUNTS	BUYBACKS	FINAL COSTS
Technical Deployment				
Servers				
Storage Platforms				
Network Connectivity				
Network Components				
Storage Network Components				
Diagnostic Workstations				
End-User Device Deployment (Tech Stations)				
Migration & Consulting Costs				
Solution Design Validation				
Site-by-Site Deployment				
Application Validation, Interface Preparation & Upgrades				
Third-Party Integrations				
Epic Corporation EHR Integration				
Microsoft Nuance PowerScribe One				
Reconfiguration of Modalities				
Migration of Study Data from Existing PACS				
TOTAL FOR SECTION				

17.4 Study-based Costs (per Study / ARC)

DESCRIPTION	MSRP/RETAIL	DISCOUNTS	BUYBACKS	FINAL COSTS
Based on Annual Volumes				
DICOM Studies				
Non-DICOM Studies				
Previous Studies Migrated from Legacy Platforms				
Internal v. External Studies				
Lifecycle Management				
TOTAL FOR SECTION				

17.5 Additional Proposed Costs

DESCRIPTION	MSRP/RETAIL	DISCOUNTS	BUYBACKS	FINAL COSTS
Additional Propose Costs (itemized in proposal)				
TOTAL FOR SECTION				

17.6 Total Proposed Costs

CATEGORIZED COSTS (ACROSS)	TOTAL NRC	TOTAL ARC	TOTAL STUDY-BASED COSTS	FINAL COSTS
PROPOSED COSTS PER CATEGORY (ITEMIZED IN PROPOSAL)				

18.0 Additional Detail

- 18.1 The vendor should document the software services cost (recurring costs) as a percentage of the list price.
- 18.2 The vendor should document the conditions of all licensing arrangements including the usual and customary pathways that clients perform upgrades v. updates to software and the costs for upgrades v. updates.
- 18.3 The vendor should indicate the timeline necessary for a rapid delivery of the products and provide information on deployment schedules based on a Letter of Intent in 3Q2025 and a solution signature on January 5, 2026.



RESPONSE LAYOUT

The layout of the document should be as follows:

- Executive Summary
- Point-by-Point Response to the RFP (incorporate Electronic Copies).
- Detailed Response
- Appendices and Attachments



CURRENT INFORMATION

Current & Future Study Volume Information

Over the course of this section, Vancouver Clinic documented the following information as to volumes and storage sizing for the environment. The VC team provided the following as part of the analysis:

- Study Volumes Actual 2021 through 2024 by Modality and Specialty.
- Study Volumes Estimated 2025 through 2028 by Modality and Specialty.

Study Volumes Actual

The chart below shows Study Volumes Actual for Calendar Year 2021 to Calendar Year 2024.

Study Type	Study Volumes Actual				Total For Four (4) Years
	CY2021	CY2022	CY2023	CY2024	
CT	18,011	17,389	19,965	20,070	75,435
DR	65,408	70,382	81,057	87,287	304,114
MR	8,930	9,885	10,839	13,431	43,085
DX	5,782	7,041	6,137	5,892	24,852
US	28,701	30,636	36,021	36,450	131,808
Echocardiology	4,766	4,326	6,337	7,601	23,030
EKG				22,530	22,530
PET	456	487	587	591	2,121
NM	2,995	2,632	3,072	3,077	11,776
C-ARM / Interventional Radiology	Included in DR volumes				
DBT	38,961	40,604	53,676	55,410	188,651
TOTALS	174,010	183,382	217,691	252,319	827,402

Study Volumes Estimated

The chart below shows Study Volumes Estimated for Calendar Year 2025 to Calendar Year 2028. This does not include new modalities being added to the environment.

Study Type	Study Volumes Actual				Total For Four (4) Years
	CY2025	CY2026	CY2027	CY2028	
CT	20,641	21,725	22,865	24,066	89,297
CR	87,975	94,651	101,855	105,596	390,077
MR	15,049	16,050	17,117	18,256	66,472
DX	6,125	7,041	7,041	7,041	27,248
US	39,645	40,482	41,336	42,208	163,671
Echocardiology	9,511	9,796	10,090	10,393	39,790
EKG	23,206	23,902	24,619	25,358	97,085
PET	591	597	603	609	2,400
NM	3,044	3,079	3,115	3,151	12,389
C-ARM / Interventional Radiology	Included in DR volumes				
DBT	56,254	57,942	59,680	61,470	235,346
TOTALS	262,041	275,265	288,321	298,148	1,123,775



APPENDIX A: ABBREVIATIONS AND TERMINOLOGY

The vendor may use the abbreviations and terminology shown below within the RFP Response, implementation process documents, drawings, and specifications:

- ACR American College of Radiology
- AD Active Directory
- ADFS Active Directory Federation Services
- AHIMA American Health Information Management Association
- AIDS Acquired Immune Deficiency Syndrome
- ANSI American National Standards Institute
- ASA American Standards Association
- ASTM American Society of Testing Materials
- BA Business Associate
- BAA Business Associate Agreement
- BTO Breast Tomosynthesis Object
- CB Scheme International Electrotechnical Commission for mutual acceptance of test reports
- CD Compact Disc
- CE Conformité Européenne
- CEHRT Certified Electronic Health Record Technology
- CFR Code of Federal Regulations
- CHPS Certified in Healthcare Privacy and Security
- CMS Centers for Medicare and Medicaid Services
- CPACS Cardiology Picture Archiving and Communication Systems
- CPHIMS Certified Professional in Healthcare Information and Management Systems
- CPOE Computerized Provider Order Entry
- CPT Current Procedural Terminology
- CSA Canadian Standards Association)
- CVIS Cardiovascular Information System
- DICOM Digital Imaging and Communications in Medicine
- DVD Digital Video Disc
- EHR Electronic Health Records
- EIA Electronic Industries Association
- EP Eligible Professional
- ePHI Electronic Protected Health Information
- ETL Electrical Testing Laboratories, Inc.
- FAQ Frequently Asked Questions
- FCC Federal Communications Commission
- FCC Class A U.S. Federal Communications Commission Declaration of Conformity, Class A
- FDA Food and Drug Administration
- FERPA Family Educational Rights and Privacy Act
- FHIR Fast Health Interoperability Resources
- FM Factory Mutual
- FR Federal Register



- GINA Genetic Information Nondiscrimination Act
- Health IT Health Information Technology
- HHS U.S. Department of Health and Human Services
- HIE Health Information Exchange
- HIMSS Healthcare Information and Management Systems Society
- HIO Health Information Organization
- HIPAA Health Insurance Portability and Accountability Act
- HITECH Health Information Technology for Economic and Clinical Health
- HIV Human Immunodeficiency Virus
- HL7 Health Level 7
- ICD-10 International Classification of Disease, 10th edition
- ICD-9 International Classification of Disease, 9th edition
- IEC International Electrotechnical Commission
- IEEE Institute of Electrical & Electronics Engineers
- IES Illuminating Engineering Society
- IOCM Imaging Object Change Management
- IPCEA International Power Cable Engineers Association
- ISTA International Safe Transit Association
- IT Information Technology
- ITIL IT Infrastructure Library
- ITSM IT Service Management
- IWA Integrated Windows Authentication
- MU Meaningful Use
- NEC National Electrical Code
- NEMA National Electrical Manufacturers Association
- NFPA National Fire Protection Association
- NIST National Institute of Standards and Technology
- NPP Notice of Privacy Practices
- NPRM Notice of Proposed Rulemaking
- OCR Office for Civil Rights
- ONC Office of the National Coordinator for Health Information Technology
- OSHA Occupational Safety & Health Administration
- PACS Picture Archiving and Communication Systems
- PCI Payment Card Industry
- PCI DSS Payment Card Industry Data Security Standard (PCI DSS)
- PCI SSC Payment Card Industry Security Standards Council (PCI SSC)
- PHI Protected Health Information
- PHR Personal Health Record
- RoHS Restriction of Hazardous Substances
- REACH Registration, Evaluation, Authorization and Restriction of Chemicals
- REC Regional Extension Center
- RIS Radiology Information System
- SRA Security Risk Assessment
- SCO Secondary Capture Object



- TIA Telecommunication Industries Association
- UL Underwriters' Laboratories, Inc.
- USC United States Code
- VNA Vendor Neutral Archive
- WEEE Waste Electrical and Electronic Equipment
- WISHA Washington Industrial Safety & Health Act

Utilize the following definition for discernment within any necessary Drawings and Specifications:

"PROVIDE" or "FURNISH" means to supply, purchase, transport, place, erect, connect, assess, and turn over to Vancouver Clinic, complete and ready for regular operation, all materials, labor, equipment, testing apparatus, controls, accessories, and all other items customarily required for the Network Infrastructure System.

"SUPPLY" means to purchase, procure, acquire, and deliver complete with related accessories.

"AS DIRECTED" means as directed by Vancouver Clinic or Vancouver Clinic representative.



APPENDIX B: ACKNOWLEDGEMENT & INTENT TO PROPOSE SOLUTION

Vancouver Clinic requires the attached form for Acknowledgement & Intent to Propose Solution.



APPENDIX B: ACKNOWLEDGEMENT & INTENT TO PROPOSE

I, _____, as a Duly Authorized Representative of my organization / company, _____, (“Bidder”) with offices at _____ hereby certifies that we intend to be an active Bidder in the ENTERPRISE IMAGING RFP & SELECTION PROCESS. As a Bidder in this process, we agree to be bound by all the conditions outlined in the Master RFP Document.

This acknowledgement is effective the _____ day of _____, 2025.

Authorized Signature Section

Signature of Authorized Representative	
Printed Name of Authorized Representative	
Title of Authorized Representative	
Official Organization / Company Name	
D/B/A Name (if applicable)	
State of Incorporation	
Corporation Type	
If Public, please indicate Stock Symbol and Exchange	
Federal Tax ID Number	
Contractor License Number (if applicable)	

Vendors must notify Vancouver Clinic of their intention to bid, or not to bid, by Monday, March 17, 2025. You should use the form for your ACKNOWLEDGEMENT & INTENT TO PROPOSE. It may be copied into an email response to dreynolds@health-inovative.com. If you choose not to submit a proposal, please execute the INTENT TO WITHDRAW form.



APPENDIX C: INTENT TO WITHDRAW

Vancouver Clinic requires the attached form for Intent to Withdraw.



APPENDIX C: INTENT TO WITHDRAW

I, _____, as a Duly Authorized Representative of my organization/company, _____, (“Bidder”) with offices at _____ hereby certifies that we are withdrawing as an active Bidder in the ENTERPRISE IMAGING RFP & SELECTION PROCESS. Additionally, we hereby certify that everyone connected with our organization/company destroyed all electronic and/or printed copies of the document.

This acknowledgement is effective the _____ day of _____, 2025.

Authorized Signature Section

Signature of Authorized Representative	
Printed Name of Authorized Representative	
Title of Authorized Representative	
Official Organization / Company Name	
D/B/A Name (if applicable)	

Vendors must notify Vancouver Clinic of their intention to bid, or not to bid, by Monday, March 17, 2025. You should use the form for your ACKNOWLEDGEMENT & INTENT TO PROPOSE. It may be copied into an email response to dreynolds@health-inovative.com. If you choose not to submit a proposal, please execute the INTENT TO WITHDRAW form.



APPENDIX D: DETAILED CONTACT INFORMATION

Vancouver Clinic requires the attached form for providing Detailed Contact Information.



APPENDIX D: DETAILED CONTACT INFORMATION SHEET

Corporate Information

This is the official information for the Bidder.

Official Organization / Company Name	
D/B/A Name (if applicable)	
State of Incorporation	
Corporation Type	
If Public, please indicate Stock Symbol and Exchange	
Federal Tax ID Number	
Contractor License Number (if applicable)	

Primary Contact of Bidder

The primary contact is the representative that all primary communications for this will be coordinated through.

Name of Primary Contact	
Title of Primary Contact	
Business Unit of Primary Contact	
E-Mail Address of Primary Contact	
Mailing Address of Primary Contact	
Address Line 1	
Address Line 2	
City, State and Zip Code	
Primary Phone Number	
SMS Phone Number	
Primary Fax Number	



Executive Contact of Bidder

Use if different than primary contact of bidder. Vancouver Clinic would appreciate this information from all prospective candidates.

Name of Primary Contact	
Title of Primary Contact	
Business Unit of Primary Contact	
E-Mail Address of Primary Contact	
Primary Phone Number	
SMS Phone Number	
Primary Fax Number	

Lead Technical Contact of Bidder

Use if different than primary contact of bidder. Vancouver Clinic would appreciate this information from all prospective candidates.

Name of Primary Contact	
Title of Primary Contact	
Business Unit of Primary Contact	
E-Mail Address of Primary Contact	
Primary Phone Number	
SMS Phone Number	
Primary Fax Number	



Additional Key Contacts 1

Use if different than primary contact of bidder. Vancouver Clinic would appreciate this information from all prospective candidates.

Official Organization / Company Name	
D/B/A Name (if applicable)	
Name of Primary Contact	
Title of Primary Contact	
Business Unit of Primary Contact	
E-Mail Address of Primary Contact	
Primary Phone Number	
SMS Phone Number	
Primary Fax Number	

Additional Key Contacts 2

Use if different than primary contact of bidder. Vancouver Clinic would appreciate this information from all prospective candidates.

Official Organization / Company Name	
D/B/A Name (if applicable)	
Name of Primary Contact	
Title of Primary Contact	
Business Unit of Primary Contact	
E-Mail Address of Primary Contact	
Primary Phone Number	
SMS Phone Number	
Primary Fax Number	



Additional Key Contacts 3

Use if different than primary contact of bidder. Vancouver Clinic would appreciate this information from all prospective candidates.

Official Organization / Company Name	
D/B/A Name (if applicable)	
Name of Primary Contact	
Title of Primary Contact	
Business Unit of Primary Contact	
E-Mail Address of Primary Contact	
Primary Phone Number	
SMS Phone Number	
Primary Fax Number	

Additional Key Contacts 4

Use if different than primary contact of bidder. Vancouver Clinic would appreciate this information from all prospective candidates.

Official Organization / Company Name	
D/B/A Name (if applicable)	
Name of Primary Contact	
Title of Primary Contact	
Business Unit of Primary Contact	
E-Mail Address of Primary Contact	
Primary Phone Number	
SMS Phone Number	
Primary Fax Number	



Additional Key Contacts 5

Use if different than primary contact of bidder. Vancouver Clinic would appreciate this information from all prospective candidates.

Official Organization / Company Name	
D/B/A Name (if applicable)	
Name of Primary Contact	
Title of Primary Contact	
Business Unit of Primary Contact	
E-Mail Address of Primary Contact	
Primary Phone Number	
SMS Phone Number	
Primary Fax Number	

Additional Key Contacts 6

Use if different than primary contact of bidder. Vancouver Clinic would appreciate this information from all prospective candidates.

Official Organization / Company Name	
D/B/A Name (if applicable)	
Name of Primary Contact	
Title of Primary Contact	
Business Unit of Primary Contact	
E-Mail Address of Primary Contact	
Primary Phone Number	
SMS Phone Number	
Primary Fax Number	



APPENDIX E: DETAILED PROJECT DATES

The following attached document incorporates the Detailed Project Dates.



APPENDIX E: DETAILED PROJECT DATES

Description	Start date	End date
Preparation & Process Management		
Document Creation / Release Schedule		
Notify Initial seven (7) Bidding Entities on Pending RFP Release & Schedule	02/28/2025	02/28/2025
<i>HIMSS 2025, Las Vegas, NV - Final Determination of Bidding Entities</i>	<i>03/03/2025</i>	<i>03/06/2025</i>
Vancouver Clinic Document Sign-Off for Release of RFP	03/11/2025	03/12/2025
RFP Released to Bidding Entities and Direct Communication	03/12/2025	03/12/2025
Bid Response Management		
Formal questions period (in writing)	03/12/2025	03/24/2025
Bidders prepare RFP Responses	03/12/2025	04/02/2025
RFP Kick-Off Call with Bidding Entities	03/14/2025	03/14/2025
Notifications of "Intent" due	03/17/2025	03/17/2025
Bidding Entity 1 Conference	03/18/2025	03/18/2025
Bidding Entity 2 Conference	03/18/2025	03/18/2025
Bidding Entity 3 Conference	03/18/2025	03/18/2025
Bidding Entity 4 Conference	03/19/2025	03/19/2025
Bidding Entity 5 Conference	03/19/2025	03/19/2025
Bidding Entity 6 Conference	03/19/2025	03/19/2025
Bidding Entity 7 Conference	03/20/2025	03/20/2025
Release of Addendum to RFP (if necessary)	03/24/2025	03/24/2025
Open Conference Call Addendum to RFP	03/26/2025	03/26/2025
RFP Responses Due / Categorized and Distributed	04/03/2025	04/03/2025
Reviews, Presentations, and Demonstrations		
Review & Scoring		
Vancouver Clinic reviews RFP Responses	04/04/2025	04/10/2025
Scoring Review and Presenter Selection	04/11/2025	04/11/2025
Notification to Presenters	04/11/2025	04/11/2025
Scheduling for Onsite Presentation Visits by Vancouver Clinic	04/11/2025	04/11/2025
<i>Passover (Jewish Holidays)</i>	<i>04/12/2025</i>	<i>04/20/2025</i>
<i>Easter Sunday</i>	<i>04/20/2025</i>	<i>04/20/2025</i>
Executive / Sales Presentations (Not for Demonstrations)		
Presenting Entity 1	04/21/2025	04/25/2025
Presenting Entity 2	04/21/2025	04/25/2025
Presenting Entity 3	04/21/2025	04/25/2025
Presenting Entity 4	04/21/2025	04/25/2025
Clinical Reviews / Demonstrations Radiology		
Presenting Entity 1	04/21/2025	04/25/2025
Presenting Entity 2	04/21/2025	04/25/2025
Presenting Entity 3	04/21/2025	04/25/2025



Description	Start date	End date
Presenting Entity 4	04/21/2025	04/25/2025
Clinical Reviews / Demonstrations Mammography / Women's Imaging		
Presenting Entity 1	04/21/2025	04/25/2025
Presenting Entity 2	04/21/2025	04/25/2025
Presenting Entity 3	04/21/2025	04/25/2025
Presenting Entity 4	04/21/2025	04/25/2025
Clinical Reviews / Demonstrations Cardiology		
Presenting Entity 1 Presentation	04/21/2025	04/25/2025
Presenting Entity 2 Presentation	04/21/2025	04/25/2025
Presenting Entity 3 Presentation	04/21/2025	04/25/2025
Presenting Entity 4 Presentation	04/21/2025	04/25/2025
Final Selection		
Vancouver Clinic conducts final internal reviews	04/30/2025	04/30/2025
Vancouver Clinic I H-I-S Notifies Bidder moving to Contract	05/01/2025	05/01/2025
Contracting Process Conducted by Vancouver Clinic I H-I-S Program Complete	05/01/2025	05/01/2025



EXHIBIT A: NON-DISCRIMINATION / EQUAL OPPORTUNITY AFFIDAVIT

The following attached document must be filled out in its entirety.



EXHIBIT A: NON-DISCRIMINATION / EQUAL OPPORTUNITY AFFIDAVIT

State of _____

County of _____

_____ being first duly sworn, deposes and states that he/she is
PRINTED NAME

_____ (president, secretary, sole owner, etc.) of:
TITLE

_____, the party making the foregoing
ENTITY

proposal, and as its duly authorized representative states that such party as Bidder does not and will not discriminate against any employee or applicant for employment because of race, religion, color, sex, age, disability, military status, ancestry, or national origin. If awarded a contract under its proposal, said Bidder shall take affirmative action to ensure that applicants are employed and that employees are treated, during employment, without regard to their race, religion, color, sex, age, disability, military status, ancestry, or national origin. If successful as the Bidder under the foregoing proposal, this Bidder shall post non-discrimination notices in conspicuous places available to employees and applicants for employment, setting forth the provisions of this affidavit.

SIGNATURE

Affiant

Sworn to and subscribed before me by the above-named person this ____ day of _____,
DAY MONTH
20_____.
YEAR

NOTARY PUBLIC:

SIGNATURE

My Commission Expires:

DATE



CONTACT INFORMATION

THE FOLLOWING CONTACT INFORMATION IS PROVIDED TO IDENTIFY HEALTH INNOVATIVE SOLUTIONS & VANCOUVER CLINIC TEAM MEMBERS. THIS INFORMATION SHOULD NOT BE USED TO CONTACT ANY TEAM MEMBERS EXCEPT UNDER THE DIRECTION OF THE PROGRAM MANAGER.

H-I-S CONTACT INFORMATION

John Relic

Chief Executive Officer

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David Reynolds

Chief Innovations Officer

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(630) 841-3917 Direct/SMS
dreynolds@health-innovative.com

CLIENT CONTACT INFORMATION

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RFP Document Complete
